

DISCHARGE SUMMARY

Mrs. A Bharathi

Age/gender : 52 Years/Female
Department : Cardiology
Address : Miyapur
Hyderabad , Telangana
+91 9494329733

Admission No : IP2602082
UMR No : UMR49441
Ward : ICU/ICU-2
Referred By:

Admitted 18-Aug-2026

Discharged 20-Aug-2026

Consultants:

Dr. M.S.S Mukharjee MD,DM(Card), DNB(Card)
Dr. Movva Srinivas MD,DM (NIMS)
Dr. Vinod K Unni MD,DM (Cardiology)
Dr. R V Venkat Rao MD,DM (NIMS)
Dr. T. Vamshidhar MBBS,DNB (CTVS)
Dr. Vikash Kumar Shukla MBBS, P.G.D.C.O (Cardiologist)



Diagnosis:

CAD / NSTEMI
GOOD LV FUNCTION
CAG - MODERATE CAD (19/08/26)
ADVICE : MEDICAL MANAGEMENT



Risk Factors / Past History:

HYPERTENSION
TYPE 2 DM
HYPOTHYROID
BREAST CANCER (S/P CHEMOTHERAPY) - 2002
ENDOMETRIAL CANCER (S/P CHEMOTHERAPY) - 2019
S/P HYSTERECTOMY



Treatment Plan:

MEDICATIONS AS ADVISED

For Appointment
+91 88866 88063

For Emergency
+91 77997 88879

📍 Muncipal Road, 500011, Hyderabad, India
📞 08866688063

Chief Complaints or symptoms:

The patient presented with complaints of chest discomfort since this morning, associated with bilateral shoulder pain, retrosternal chest pain, dyspnea on exertion, and neck pain. In view of these concerning symptoms, admission was advised for further evaluation and management.

General Examination:

- TEMP-Normal
- BP-150/90MMHG
- PR-78/min
- SPo2-98% on RA
- RR-20/min
- CVS-S1S2 +
- RS-BAE+
- CNS-NAD
- P/A-Soft

Investigation:

ENCLOSED

Course in the Hospital:

The patient was transferred to the ICCU, where he was kept under close observation and placed on continuous cardiac monitoring. A 2D echocardiogram revealed a poor acoustic window, with no left ventricular regional wall motion abnormalities. Concentric left ventricular hypertrophy (LVH) and Grade I left ventricular diastolic dysfunction were noted. Left ventricular and right ventricular systolic functions were good. There was no evidence of pulmonary embolism or intracardiac thrombus.

Subsequently, the patient was taken to the catheterization laboratory, where a coronary angiogram was performed via the right femoral artery approach. The angiogram revealed moderate coronary artery disease (CAD). In view of the angiographic findings, medical management was advised. The procedure was completed successfully without any complications.

During the hospital stay, the patient was treated with antiplatelet agents, statins, antiemetics, antacids, and other supportive cardiac medications as indicated. The patient remained hemodynamically stable and is being discharged in stable condition.



Medicines



MEDICATION	DOSAGE	FREQUENCY	TIME OF THE MEDICINE
TAB ROSUZUN GOLD	20MG	ONCE DAILY	AT 8PM AFTER FOOD
TAB TELMA-AM	40MG	ONCE DAILY	AT 8AM AFTER FOOD
TAB THYRONORM	50MCG	ONCE DAILY	AT 7AM BEFORE FOOD
TAB NONTANGLE MNT		ONCE DAILY	AT 9PM AFTER FOOD
TAB STOZINA	500MG	TWICE DAILY	AT 8AM AND 8PM AFTER FOOD
TAB NATRISE	15MG	ONCE DAILY	AT 8AM AFTER FOOD x 5 d
CAP RABIKAT DSR		ONCE DAILY	AT 7AM BEFORE BREAKFAST.
TAB DOLO	650 MG	SOS	IF FEVER/PAINS
TAB CLEZAPAMOX	625mg	TWICE DAILY	At 8am & 8pm After food x 5 d

CONTINUE OLD DIABETIC MEDICATIONS AS EARLIER

Note & Instructions:

REVIEW AFTER 1 WEEK WITH CBP, ECG, SR. CREATININE, SR. ELECTROLYTES, RBS REPORTS IN OPD WITH
Dr. M.S.S. MUKHARJEE (CARDIOLOGY)

When And How To Obtain:

IF ANY EPISODE OF

1. SHORTNESS OF BREATH
2. CHEST PAIN
3. PANIC ATTACK / SYNCOPAL ATTACK

IN EMERGENCY CONTACT IMMEDIATELY DR. KRANTHI KUMAR -9949996644
FOR APPOINTMENTS CALL 8886688063 MIYAPUR (9:30AM TO 3PM)



Medical Officer

Created By
Modified By

DUTY DOCTOR
DUTY DOCTOR


Dr. M.S.S. MUKHARJEE
MD, DM, (CARDIOLOGY)
Sr. CONSULTANT CARDIOLOGIST